OSCE Evaluation Report

Rutgers Robert Wood Johnson Medical School

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Overall Performance

Metric	Score	Status
Critical Actions	40%	Fail
Communication & Empathy	88%	Pass
Overall Result		**FAIL**

Summary Feedback

The student demonstrated strong interpersonal skills and established good rapport with the patient. The history taking was generally comprehensive, though the clinical reasoning process could be improved.

Strengths:

- [00:00:04] - Demonstrated excellent communication skills and empathy.
- [00:08:59] - Summarized the patient's history effectively.
- [00:10:39] - Explained next steps and encouraged shared decision-making.

Areas for Improvement:

- [00:04:19] - Needs to be more explicit about differential diagnoses and reasoning.
- [00:05:08] - Could explore potential red flag symptoms more thoroughly.
- [00:05:14] - Should ensure all relevant data is gathered, including full medication and allergy history.

Summary:

The student excelled in communication and empathy. Focus should be placed on developing more explicit clinical reasoning skills, particularly hypothesis generation and differential diagnosis development.

Critical Action Checklist

Hypothesisgeneration

✓ [00:00:22] STUDENT: Um, can you tell me why you're here today?

Student initiates encounter by asking the chief complaint, setting the stage for potential hypothesis generation.

✓ [00:00:26] STUDENT: Is there anything else besides your headache that you want to address here today?

Student attempts to broaden the differential early by asking about other concerns.

✗ [00:01:24] STUDENT: So tell me a little bit more about this the head headache that you're having.

No specific system hypotheses explicitly voiced; questioning is general.

✓ [00:03:31] PATIENT: Uh it's kind of all over. Um, but it it also I've got this shooting pain down my neck and my neck is is very stiff.

The patient mentions neck stiffness, implicitly suggesting a musculoskeletal or neurological cause that student then explores in later questions.

✗ No statements present.

Student does not mention any initial broad hypotheses such as migraine vs. tension headache vs. other pathology.

Datagathering

✓ [00:01:31] PATIENT: Well, um, it started about three days ago and um, uh, nothing has helped it.

Patient's narrative provides further detail after initial questioning.

✓ [00:02:33] STUDENT: Can you tell me, uh, anything that makes it better? Is there anything?

Student uses OPQRST framework by asking about alleviating factors.

✓ [00:02:52] STUDENT: Is there anything that makes it worse?

Asks about aggravating factors, continuing OPQRST.

✓ [00:05:14] STUDENT: I would like to go some talk about a little bit about your medical history and your social history.

Broadens data gathering with relevant background information.

✗ No statement present.

Although past medical history is explored, medication history is not thoroughly investigated (only Tylenol/Motrin). No allergy history is asked.

Problemrepresentation

✓ [00:08:59] STUDENT: Let me just summarize to make sure I've got everything straight so far is um, you started having the onset of a severe headache ab

Student offers a summary of findings, which is a positive step.

✓ [00:09:02] STUDENT: it's worse with movement. Light really makes it hurt bad.

Accurately restates patient's description of aggravating factors.

✓ [00:09:12] STUDENT: Medical history, you're in hyper, you have high blood pressure, but that's controlled with diet.

Incorporates relevant medical background into the summary.

✓ [00:09:25] STUDENT: Impacting your day-to-day living daily life.

Summarizes the impact of the problem on the patient's life.

✗ No statement present.

The summary lacks explicit confirmation from the patient that it is accurate or complete, though implied through the conversation flow.

Differentialdiagnosis

✗ No statement present.

Student does not explicitly list or prioritize any differential diagnoses.

✗ No statement present.

Justification for potential diagnoses is missing, as no diagnoses are explicitly mentioned.

✗ No statement present.

Student did not explicitly address red flag symptoms (e.g. fever, vision changes, weakness) which limits assessment of considering serious causes.

✗ No statement present.

The student did not acknowledge the patient's concern about a brain tumor as a possible serious cause.

✗ No statement present.

There is no discussion of next steps beyond physical exam to narrow differential diagnosis or consider serious causes.

Communication & Empathy Assessment

Fosteringrelationship

✓ Sets stage for the encounter and patient comfort

[00:00:04] STUDENT: Hi, I'm Toni, um Harris. I'll be your doctor today.

✓ Listens actively and conveys understanding

[00:04:46] STUDENT: I can understand your concern.

✓ Shows care and compassion

[00:02:04] STUDENT: I can't dim the lights but I will be quick. Would it be ok if I took some notes?

Gatheringinformation

✓ Encourages patient to share openly, avoids interruption, uses reflective prompts

[00:01:24] STUDENT: Tell me a little bit more about this headache that you are having.

✓ Avoids interrupting the patient

Throughout the interview, the student lets the patient complete her thoughts without interruption.

Providinginformation

✓ Adjusts explanations to patient's knowledge level; confirms comprehension

[00:00:40] STUDENT: What we can do is while we are talking and doing the history and physical I will have my office secretary look into the insurance

Helpingpatientdecisions

✓ Encourages shared decision-making, asks for patient preferences, involves them in next steps

[00:10:39] STUDENT: What I would like to do is complete a physical exam and do some testing.

✓ Asks the patient if they have any questions or other concerns

[00:10:44] STUDENT: Are there any other concerns I need to address before we get to the final exam?